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A RadNet Imaging Center

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EXAM: MRI PELVIS WITHOUT AND WITH CONTRAST

HISTORY: Suspected ankylosing spondylitis, evaluation for sacroiliitis.

TECHNIQUE: Multiple MRI sequences of the sacroiliac joints were obtained on a 3.0 Tesla scanner with and without intravenous contrast.

Contrast: The patient was injected with 10 cc Clariscan from a 10 cc single-use vial.

COMPARISON: None available.

FINDINGS:

*SI JOINTS-*

Active disease: Mild ill-defined subchondral bone marrow T2 hyperintensity and enhancement in the bilateral sacroiliac joints. No focal erosion is identified.

Structural change: No evidence of fatty marrow infiltration, sclerosis, or ankylosis in the upper and lower aspects of the bilateral sacroiliac joints.

Other inflammatory change: No evidence of sacroiliac joint effusions.

MISCELLANEOUS-
-Normal signal and morphology of the visualized muscular structures.
-Trace free fluid in the dependent portion of the pelvis. Ovoid T2 hyperintensity in the right adnexa measuring up to 1.7 cm may represent a dominant follicle or a small cyst.

IMPRESSION:

1. Mild ill-defined subchondral bone marrow T2 hyperintensity and enhancement in the bilateral sacroiliac joints. This is most consistent with acute symmetric sacroiliitis.

2. No structural changes in the sacroiliac joints to suggest chronic sacroiliitis.



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